CRESCENT PAIN & SPINE
Interventional Pain Medicine · 41 Crescent Ct, Riverton · (555) 010-6900 · Fax (555) 010-6901

Omar Haddad, MD
Procedure Center

PROCEDURE NOTE

Cervical Interlaminar Epidural Steroid Injection under Fluoroscopy

PATIENT		DOB	MRN
DOE, JOHN		08/14/1991	CPS-60412
DATE OF SERVICE	PROCEDURE START / END	PHYSICIAN	
05/15/2026 (Fri)	09:52 / 10:06	Omar Haddad, MD	
REFERRING		PAYER	
Alana Reyes, MD, Lakeside Orthopedics (03/27/2026)		Peach State Health Plan — IN NETWORK · PSH-55017742 · GRP-41208 · auth PA-26-118734	

Pre-procedure diagnosis: Cervical disc herniation C5-C6 with right C6 radiculopathy (M50.122), onset after motor vehicle collision 02/06/2026. MRI 03/13/2026 (Summit Imaging): C5-C6 right paracentral herniation with mild right foraminal narrowing. Persistent right arm paresthesia despite 11 weeks of chiropractic/PT, NSAID and gabapentin.
Post-procedure diagnosis: Same.

Procedure: Cervical epidural steroid injection, interlaminar approach, **C6-C7, right paramedian**, with fluoroscopic guidance and epidurography (CPT 62321).

Consent: Risks (infection, bleeding, headache, dural puncture, nerve injury, increased pain, steroid effects, rare spinal cord injury), benefits and alternatives were discussed. Patient's questions answered; written consent signed 09:31. No anticoagulants or antiplatelet agents; no fever or active infection. Pre-procedure pain 6/10 (arm paresthesia, neck). Time-out performed 09:50: correct patient, procedure, level and side confirmed.

Description: Patient placed prone with the neck flexed on a cushion. Pre-procedure vitals BP 128/80, HR 74, SpO2 98% RA. The posterior neck was prepped with chlorhexidine and draped sterilely. Under AP fluoroscopy the C6-C7 interlaminar space was identified and the skin anesthetized with 1% lidocaine, 2 mL. A 20-gauge 3.5-inch Tuohy needle was advanced to the right of midline under intermittent AP and contralateral oblique views, using loss-of-resistance to preservative-free saline. Epidural space entered at 09:58. After negative aspiration for blood and CSF, 1.0 mL of iohexol 240 showed epidural spread cephalad to C5-C6 with right-sided predominance, no vascular uptake and no intrathecal pattern. Injectate: **dexamethasone sodium phosphate 10 mg (preservative-free) with 2 mL preservative-free normal saline**, injected slowly with no paresthesia. Needle removed intact; bandage applied.

Sedation: none (patient declined). **Estimated blood loss:** none. **Fluoroscopy time:** 24 s. **Complications:** none.

Tolerance: Tolerated well. Post-procedure vitals at 10:20 BP 124/78, HR 70, SpO2 99%. Neurologically intact upper and lower extremities; ambulating without difficulty. Post-procedure pain 2/10. Discharged 10:40 in stable condition with an adult driver.

Post-procedure plan:

1. Discharge instructions reviewed and given in writing: no driving today; ice to the site; light activity only through the weekend. Work note given: no heavy lifting or warehouse work — off work 05/15/2026 through 05/18/2026; may return 05/19/2026. Call for fever, severe headache when upright, new weakness or numbness.
2. Hold chiropractic manipulation for 72 hours; physical therapy may resume after 72 hours.
3. Rx: methylprednisolone 4 mg dose pack (6-day taper), #21 — fill and hold; start only for a post-injection radicular flare in the first 72 hours, after calling this office.
4. Continue gabapentin and meloxicam as prescribed by Dr. Reyes.
5. Expect benefit over 3–14 days. Repeat injection may be considered if relief is less than 50% or symptoms recur. Report response to Dr. Reyes; return here as needed.

Omar Haddad, MD

Omar Haddad, MD · electronically signed 05/15/2026 10:48 · cc: Alana Reyes, MD, Lakeside Orthopedics